

1. Administrative & Study Identification

Full Study Title: Phase 2/3 Trial of Izalontamab Brengitecan vs Platinum-based Chemotherapy for Metastatic Urothelial Cancer With Disease Progression on or After Immunotherapy **Short Title/**
Acronym: IZABRIGHT-Bladder01 **Protocol Number:** CA244-0012 **NCT**
Number: NCT07106762 **Posting ID:** 494751493125796818 **Trial Status:**
RECRUITING (Currently recruiting participants) **Sponsor Name:**
Bristol Myers Squibb **Company:** Bristol Myers Squibb
Investigational Product: Izalontamab Brengitecan (Iza-bren; BL-B01D1; BMS-986507) **Phase of Development:** PHASE2, PHASE3 (Criteria text: Trial must be in PHASE2, PHASE3) **Medical Monitor:** BMS Clinical Trials Contact Center

2. Study Synopsis & Rationale

2.1 Study Objective Primary Outcomes: * Phase 2: Recommended Phase 3 Dose (RP3D) of BMS-986507 (Izalontamab Brengitecan). * Phase 3: Progression-Free Survival (PFS) assessed using Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1) per blinded independent central review (BICR). * Phase 3: Overall Survival (OS).

Secondary Outcomes: * Phase 2: Objective Response (OR) * Phase 2: Progression-Free Survival (PFS) * Phase 2: Duration of Response (DOR) * Phase 3: Objective Response Rate (ORR), Duration of Response (DOR), and time to definitive deterioration (per EORTC QLQ-C30).

2.2 Background & Rationale Urothelial cancer is a type of malignancy originating from the urothelial cells that line the urinary tract. While most frequently found in the bladder, it can also develop in the renal pelvis, ureters, and urethra, representing the uncontrolled growth of these specialized epithelial cells. While Antibody-Drug Conjugate (ADC) combined with immunotherapy has become a transformative first-line therapy for advanced urothelial cancer (aUC), treatment options remain limited for patients who progress on or after standard-of-care immunotherapy-based therapies. Izalontamab brengitecan is a potentially first-in-class ADC composed of an EGFR x HER3 bispecific antibody, conjugated to a novel topoisomerase I inhibitor payload, which induces tumor cell death via antibody-dependent cellular cytotoxicity and inhibition of EGFR/HER3 signaling. This study aims to provide an alternative for heavily pre-treated patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: INTERVENTIONAL **Control Type:** Active Comparator (Platinum-based Chemotherapy: cisplatin [Drug preferred name: cisplatin, ATC code: L01XA01, Trade names: Platinol] or Carboplatin + Gemcitabine) **Blinding:** Open-label **Randomization:** Randomized (1:1 ratio in Phase 3)

3.2 Planned Enrollment & Duration Targeted Enrollment: 470 participants **Number of Sites:** 98 locations globally **Estimated Study Start:** 30-Sep-2025 **Estimated Primary Completion:** 26-Jan-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants must have histologically confirmed advanced urothelial carcinoma. 2. Age ≥ 18 years. 3. Participants must be eligible to receive platinum-based chemotherapy. 4. Participants must be Anti-PD-(L)1-experienced (in locally advanced or metastatic setting), either in combination with or sequential to another systemic therapy. 5. Participants treated only in the peri-operative setting must have relapsed within 12 months of the last dose of the treatment. 6. Participants must have ≥ 1 measurable lesion per RECIST v1.1. 7. Participants must have Eastern Cooperative Oncology Group (ECOG) Performance Status 0-1.

4.2 Exclusion Criteria 1. Participants must not have platinum-based chemotherapy exposure within 12 months. 2. Participants must not have received >2 prior regimens irrespective of the setting. 3. Participants must not have prior ADC therapy targeting EGFR or HER3. 4. Participants must not have prior therapy with topoisomerase 1 inhibitor. 5. Participants must not have active, untreated brain metastases. 6. Other protocol-defined inclusion/exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Izalontamab Brengitecan administered at the determined Recommended Phase 3 Dose (RP3D). **Route of Administration:** Intravenous. **Duration of Treatment:** Treatment continues until disease progression or unacceptable toxicity. Note: Platinum-based chemotherapy (comparator arm) is limited to a maximum of 6 cycles.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing chemotherapy-associated toxicities. **Prohibited:** Concurrent investigational agents, topoisomerase 1 inhibitors, and other targeted ADC therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging (RECIST v1.1)
Baseline	Day 0	Randomization, First Dose Administration
Interim	Q3W/Q4W	Safety Labs, Efficacy Assessment (Tumor Scans), Quality of Life forms (EORTC QLQ-C30)
End of Study	Variable	End of Treatment (Progression/Toxicity), Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: * Phase 2: Recommended Phase 3 Dose (RP3D) of BMS-986507. * Phase 3: Progression-Free Survival (PFS) and Overall Survival (OS). **Measurement Method:** PFS assessed using RECIST v1.1 per blinded independent central review (BICR).

7.2 Secondary & Exploratory Endpoints * Phase 2: Objective Response (OR) * Phase 2: PFS * Phase 2: Duration of Response (DOR) * Phase 3: Objective Response Rate (ORR), Duration of Response (DOR), and time to definitive deterioration in HRQoL (per EORTC QLQ-C30). All assessed by BICR.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of toxicities through patient clinical visits and laboratory assessments. **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness to the sponsor. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee instituted to review safety and clinical efficacy during the Phase 2 to Phase 3 transition.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints (PFS and OS); Safety Set for all treatment-emergent adverse event analyses. **Sample Size Justification:** 470 participants targeted to provide sufficient power to detect a significant improvement in PFS and OS. **Handling of Missing Data:** Standard survival analysis methodologies (e.g., right-censoring for PFS/OS in patients lost to follow-up).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring to ensure

protocol adherence. **Audit Trail:** Robust eCRF locking, source data verification, and central tracking of BICR imaging assessments.

11. Study Identifiers & Governance

Primary ID: CA244-0012 **Registry Number:** NCT07106762 **Posting ID:** 494751493125796818 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** IZABRIGHT-Bladder01 **Official Regulatory Title:** A Randomized, Open-label, Phase 2/3 Trial of Izalontamab Brengitecan Versus Platinum-based Chemotherapy for Metastatic Urothelial Cancer in Participants With Disease Progression on or After an Immunotherapy-based Treatment

12. Clinical Framework (Oncology Specific)

Disease Areas: Urothelial Cancer **Primary Condition:** Metastatic Urothelial Carcinoma (Preferred Name: Suspected urothelial cancer) **Semantic Type:** Finding **MeSH Code:** D001749 **SNOMED ID:** 424564007 (Hierarchy: 18873004 | Malignant neoplasm of urinary tract | Malignant neoplasm of urinary tract (disorder)) **Diagnosis Threshold:** Histologically confirmed advanced urothelial carcinoma with ≥ 1 measurable lesion per RECIST v1.1 **Medical Methodology:** Bispecific Antibody-Drug Conjugate (ADC) vs. standard Platinum-based Chemotherapy **Optimization Target:** Disease progression control, overall survival prolongation, and preservation of quality of life

13. Study Procedures & Methodology

Management Pathway: Treat until progression or unacceptable toxicity (limited to 6 cycles for the chemotherapy control arm) **Education Protocol (HCP):** Protocol specific training, focusing on RECIST v1.1 assessment and ADC toxicity management **Education Protocol (Patient):** Informed consent, potential side effects of Topoisomerase I inhibitors (e.g. Ed-04 payload) **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all primary and secondary tumor response endpoints

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until study completion (Estimated Nov 2033) **Onsite Visit Cadence:** Aligned with treatment cycles **Remote Monitoring Frequency:** Between treatment cycles as necessary for safety **Checklist Review Interval:** Regular tumor imaging assessments per RECIST v1.1 guidelines

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					